

## **1. Administrative & Study Identification**

**Full Study Title:** DESTINY-Endometrial01: An Open-Label, Sponsor-Blinded, Randomized, Controlled, Multicenter, Phase III Study of Trastuzumab Deruxtecan (T-DXd) Plus Rilvegostomig or Pembrolizumab vs Chemotherapy Plus Pembrolizumab as First-Line Therapy of HER2-Expressing (IHC 3+/2+), Mismatch Repair Proficient (pMMR), Primary Advanced or Recurrent Endometrial Cancer **Short Title/Acronym:** DESTINY-Endometrial01 (also GOG-3098, ENGOT-EN24) **Protocol Number:** D781DC00001 **NCT Number:** NCT06989112 **Posting ID:** 205603543587296869 **Trial Status:** Recruiting **Sponsor Name:** AstraZeneca (in collaboration with Daiichi Sankyo) **Investigational Product:** Trastuzumab deruxtecan (T-DXd / Enhertu) + Rilvegostomig (AZD2936) OR Pembrolizumab (Keytruda) **Phase of Development:** Phase III **Medical Monitor:** Dr. Vicky Makker, Memorial Sloan Kettering Cancer Center (Contact: 646-888-4224) / AstraZeneca Clinical Operations Lead

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## **2. Study Synopsis & Rationale**

**2.1 Study Objective Primary Objective:** To investigate the efficacy of first-line T-DXd + rilvegostomig (Arm A) and/or T-DXd + pembrolizumab (Arm B) when compared to the standard of care chemotherapy (carboplatin + paclitaxel) + pembrolizumab (Arm C) by assessing Progression-Free Survival (PFS) via Blinded Independent Central Review (BICR). **Secondary Objectives:** To evaluate Overall Survival (OS), Progression-Free Survival (PFS) assessed by investigator, Time from randomization to second progression or death (PFS2), Objective Response Rate (ORR), Duration of Response (DOR), and the safety and tolerability profiles of the treatment regimens.

**2.2 Background & Rationale** Endometrial cancer incidence and mortality rates are rising, and primary advanced or recurrent disease carries a poor prognosis (median OS up to 30 months). Currently, there are no FDA-approved HER2-directed therapies in the first-line setting for this disease. Based on robust monotherapy efficacy shown in the DESTINY-PanTumor02 trial, combining the HER2-directed antibody-drug conjugate (ADC) T-DXd with immunotherapy (rilvegostomig or pembrolizumab) represents a promising approach to improve response rates and PFS in patients with HER2-expressing, mismatch repair-proficient (pMMR) tumors.

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### 3. Study Design & Methodology

**3.1 Design Framework** Study Type: Interventional Control Type: Active Comparator (Chemotherapy + Pembrolizumab) **Blinding:** Open-label, Sponsor-Blinded **Randomization:** Randomized, Parallel Assignment

**3.2 Planned Enrollment & Duration** Target **N**: Approximately 600 evaluable patients **Number of Sites:** Multicenter, global (North America, South America, Australia, Europe, and Asia) **Estimated Study Start:** 27-Mar-2025 **Estimated Primary Completion:** 19-Jan-2029

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### 4. Subject Selection (Eligibility Criteria)

**4.1 Inclusion Criteria** 1. Female patients  $\geq 18$  years of age. 2. Histologically confirmed diagnosis of epithelial endometrial carcinoma (all histologies allowed except sarcomas; carcinosarcomas are allowed). 3. Following surgery or diagnostic biopsy, patient must have primary advanced disease (Stage III/IV) or first recurrent endometrial cancer. 4. Endometrial cancer with HER2 IHC expression of 3+ or 2+ as assessed by prospective central testing. 5. Endometrial cancer that is mismatch repair proficient (pMMR) by prospective central IHC testing.

**4.2 Exclusion Criteria** 1. Prior anti-cancer medication for advanced cancer or prior immunotherapy (prior neoadjuvant/adjuvant chemotherapy allowed if recurrence occurred  $\geq 6$  months post-treatment). 2. History of (non-infectious) Interstitial Lung Disease (ILD)/pneumonitis requiring steroids, current ILD/pneumonitis, or underlying severe pulmonary disorder (e.g., severe asthma, COPD, pulmonary emboli). 3. Medical history of myocardial infarction within 6 months, symptomatic congestive heart failure (NYHA Class II-IV), or clinically significant arrhythmia. 4. Uncontrolled HIV, or chronic/active hepatitis B or C (unless receiving suppressive antiviral therapy with normal ALT and controlled viral load). 5. Spinal cord compression or clinically active central nervous system (CNS) metastases.

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### 5. Intervention & Treatment Plan

**5.1 Investigational Regimen Dosage/Frequency:** \* **Arm A:** Trastuzumab deruxtecan (T-DXd) + Rilvegostomig administered every 3 weeks (Q3W). \* **Arm B:** Trastuzumab deruxtecan (T-DXd) + Pembrolizumab administered Q3W. \* **Arm C (Control):** Carboplatin (Paraplatin) + Paclitaxel + Pembrolizumab administered Q3W for 6 cycles, followed by maintenance Pembrolizumab Q6W. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Treatment continues until disease progression, unacceptable toxicity, or up to 20 total cycles (approx. 24 months) for maintenance therapy.

**5.2 Concomitant & Prohibited Therapy Permitted:** Concurrent use of hormonal therapy for non-cancer-related conditions (e.g., HRT). **Prohibited:** Any concurrent anticancer treatment without an adequate washout period prior to the first dose of study intervention.

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**6. Study Timeline & Visit Schedule**

|              |                     |                                                                                                 |
|--------------|---------------------|-------------------------------------------------------------------------------------------------|
| Visit ID     | Window (Days)       | Key Activities/Procedures                                                                       |
| Screening    | Day -28 to 0        | Informed Consent, Medical History, Tumor Biopsy for Central HER2/pMMR Testing, Baseline Imaging |
| Baseline     | Day 0               | Randomization, First IV Infusion Dose                                                           |
| Treatment    | Q3W (Every 21 days) | IV Infusions, Safety Labs, Efficacy Assessment, AE Monitoring                                   |
| End of Study |                     |                                                                                                 |
| Follow-Up    |                     | Final Vital Signs, Exit Interview, Survival and Progression tracking                            |

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**7. Outcome Measures (Endpoints)**

**7.1 Primary Endpoint Definition:** Progression-Free Survival (PFS) defined as the time from randomization until objective tumor progression or death due to any cause. **Measurement Method:** Assessed via Blinded Independent Central Review (BICR) per Response Evaluation Criteria in Solid Tumors, version 1.1 (RECIST 1.1).

**7.2 Secondary & Exploratory Endpoints** \* Overall Survival (OS) \* Progression-Free Survival (PFS) as assessed by the investigator \* Time from randomization to second progression or death (PFS2) \* Objective Response Rate (ORR) assessed by BICR and investigator \* Duration of Response (DOR)

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**8. Safety & Adverse Event Reporting**

**Adverse Event (AE) Monitoring:** Continuous evaluation of toxicity, with a specific focus on monitoring pulmonary symptoms indicative of drug-induced ILD/pneumonitis and cardiotoxicity. **Serious Adverse Events (SAEs):** Standard 24-hour reporting timeline to the sponsor and Institutional Review Board (IRB). **Data Monitoring Committee (DMC):** External Independent Data Monitoring Committee (IDMC) to review safety and efficacy signals throughout the trial.

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**9. Statistical Analysis Plan (SAP) Summary**

**Analysis Populations:** Intent-to-Treat (ITT) population for primary efficacy (PFS) and OS evaluations. Safety Set for safety and tolerability. **Sample Size Justification:** Target \$N = 600\$

adequately powered to detect statistically significant improvements in PFS when comparing the experimental arms (A and B) against the active comparator arm (C). **Handling of Missing Data:** Standard oncology guidelines incorporating right-censoring methods (Kaplan-Meier estimates) for time-to-event efficacy endpoints.

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## 10. Quality Assurance & Regulatory Compliance

**GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Combination of routine onsite monitoring visits and centralized remote data monitoring by the sponsor. **Audit Trail:** Validated eCRF platforms with query generation for data clarification forms; strict central laboratory chain of custody for HER2/pMMR biopsies.

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## 11. Study Identifiers & Governance

**Primary ID:** D781DC00001 **Registry Number:** NCT06989112 **Posting ID:** 205603543587296869 **Trial Status:** Recruiting **Sponsor/Lead Organization:** AstraZeneca **Study Title:** DESTINY-Endometrial01: A Phase III Study of Trastuzumab Deruxtecan Plus Rilvegostomig or Pembrolizumab as First-Line Treatment of HER2-Expressing (IHC 3+/2+), Mismatch Repair Proficient (pMMR) Endometrial Cancer **Official Regulatory Title:** DESTINY-Endometrial01: An Open-Label, Sponsor-Blinded, Randomized, Controlled, Multicenter, Phase III Study of Trastuzumab Deruxtecan (T-DXd) Plus Rilvegostomig or Pembrolizumab vs Chemotherapy Plus Pembrolizumab as First-Line Therapy of HER2-Expressing (IHC 3+/2+), Mismatch Repair Proficient (pMMR), Primary Advanced or Recurrent Endometrial Cancer

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## 12. Clinical Framework (Endometrial Cancer Specific)

**Primary Condition:** Primary Advanced or Recurrent Endometrial Cancer (Endometrial Carcinoma) **Diagnosis Threshold:** HER2-Expressing (IHC 3+/2+) and Mismatch Repair Proficient (pMMR) **Medical Methodology:** Targeted Antibody-Drug Conjugate (ADC) combined with Immunotherapy (PD-1/TIGIT bispecific or PD-1 inhibitor) **Optimization Target:** First-line Progression-Free Survival enhancement

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## 13. Study Procedures & Methodology

**Management Pathway:** First-line systemic oncology clinical pathway for advanced EC **Education Protocol (HCP):** Strict guidance on early identification, management, and reporting of T-DXd-related

Interstitial Lung Disease (ILD) / Pneumonitis **Education Protocol (Patient)**: Onsite education regarding self-monitoring for pulmonary symptoms (cough, dyspnea, fever) and infusion reactions. **Quality Audit Mechanism**: Blinded Independent Central Review (BICR) for all RECIST v1.1 tumor imaging

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## 14. Observation & Follow-Up Schedule

**Total Duration**: Estimated 24 months of active treatment + Long-term survival follow-up **Onsite Visit Cadence**: Every 3 Weeks (Q3W) correlating with IV infusion cycles **Remote Monitoring Frequency**: Inter-cycle telehealth or symptom checking as clinically indicated. **Checklist Review Interval**: Radiographic tumor assessments initially every 6 to 9 weeks per RECIST 1.1 protocol guidelines.

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## 15. Sign-off & Approval

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|-------------------------------|---------------------|-----------|------------------------|---------------------------------|----------------|-------|------------------------|
| Role                          | Name                | Signature | Date                   | :---                            | :---           | :---  | :---                   |
| <b>Principal Investigator</b> | [Investigator Name] | _____     | [DD- <b>MMM-YYYY</b> ] | <b>Clinical Operations Lead</b> | [Sponsor Name] | _____ | [DD- <b>MMM-YYYY</b> ] |
| <b>Lead Statistician</b>      | [Statistician Name] | _____     | [DD- <b>MMM-YYYY</b> ] |                                 |                |       |                        |